EXAM 1

RECALL

DECEMBER 2020

1- **concerning ureter which is true?**

a) has constriction when enter pelvic prim
b) mainly supply by left renal artery
c) inferior epigastric vein run on lateral side
d) run in direct contact with the tip of transverse process of the lumber vertebra

2- **female 65 years diagnosed with pelvic tumor metastasis to medial inguinal lymph nodes what is the origin?**

a) A-urinary bladder
b) B-ovaries
c) C -uterus
d) D-uterine tube

3-24 years got trauma to pelvis and penis come with swollen in scrotum the fluid extravasated to:

a) A-deep perineal pouch
b) B-tunica vaginalis
c) C-douglas pouch
d) D -superficial perineal pouch

great saphenous pass posterior to the medial malleolus

4-small(short) saphenous vein which is correct?

a) A-ascend up on medial malleolus — anterior to lateral malleolus
b) B-always has valve in terminal end (12 valves)
c) C-has relation with lateral sural nerve — in lower calf only
d) D -perforate deep fascia in upper part of popliteal fossa — superficial not deep

5 - In exploration popliteal artery?

a) run anterior to sural nerve
b) B-run superior to popliteal vein — A is anterior to the vein
c) C -anterior tibial artery is Infront of popliteal muscle in high division — low division
d) D-go round and flat to words knee to be ant tibial artery — ant and post

6- **Lymph node of trigone of bladder drain to?**

A- internal iliac lymph nodes — prostate Ca + cervix Ca + lower rectum + upper vagina
B- external iliac lymph nodes — upper rectum to IM LN
C- Internal and external iliac lymph nodes — upper uterus + bladder Ca

D- Superficial inguinal lymph nodes lower anal canal + lower vagina + skin

E- D-paraaortic lymph nodes fundus of uterus + Ovary + fallopian tube

7- Which of the following concerning the penis is incorrect?

A- A-glans of penis form bulbospongiosus

B- B - ischiocovernosus cover the bulp of penis bulb covered by bulbospongiosus
cover corpora cavernosa > cause erection

C- C-penile urethra lie in bulbospongiosus

D- D-labia majora is bounded the vestibule

8- Which of the following concerning the perineum is incorrect?

A- A-somatic sensory of clitoris and glans of penis come from internal pudendal nerve

B- B-erection of penis and clitoris by parasympathetic

erection by para
ejaculation by sym

C- C-contraction of pulbospongiosus and covernousus muscles by parasympathetic

D- D -bulpspongiousus cover RT and LEFT crura of clitoris

9- Internal spermatic fascia made from?

A- A-external oblique muscle external spermatic facia

B- B-internal oblique muscle

C- C-cremastric facia internal oblique M

dartos fascia in scrotum
Scarpa's fascia

D- D -transversalis fascia

10 - Concerning surgery the sciatic nerve?

A- A-come anterior to piriformis sciatic N is inferior to piriformis M

B- B-has relation with superior gluteal artery

C- C-save to deal with lateral site in gluteus

11- Regarding lymph draining to lower limbs all true except?

a) A - medial group of inguinal lymph nodes is much in number than lateral group
b) B-glans of penis drain to deep inguinal lymph nodes to the superficial group
c) C-deep inguinal lymph nodes is fewer in number
d) popliteal glands drain entirely to the deep group of inguinal lymph nodes

12. Which of the following ligament attach to uterus and have peritoneum?

A- cardinal ligament

A- B -broad ligament

B- C-round ligament

C- D-suspensory ligament

broad ligament contains the following structures:
- Fallopian tubes.
- Ovaries.
- Ovarian arteries.
- Uterine arteries.
- Round ligaments.
- Suspensory (infundibulopelvic) ligaments.
- Epoophoroon

13. Following statement is incorrect about perineum structure:

a. A -anorectal ring formed by subcutaneous, superficial and 2deep part of external anal sphincter = Deep part of External Sphincter + Puborectalis muscle

b. B. perineal membrane attach laterally to pubic arch and ischial spine.

14. cancer from pelvic viscera can deposits cancerous cells in the vertebrae, this communication occurs between the internal iliac veins and vertebral veins through:

a. lateral sacral veins

b. lumbar vein

c. umbilical veins

d. prostatic plexus

15. Concerning pelvic which is true? except

a) Femoral ring is 1.25 inch lateral and below the pubic tubercle

b) B-The anterior superior iliac spine is at spinous level of second sacral vertebra L 4 -5 iliac crest L4

c) The greater sciatic notch lies at the level of third sacral vertebra

d) D-The line that meet the anterior superior iliac spine with most prominent point in ischial tuberosity pass directly through the acetabulum Roser-Nélaton line: for hip dislocation

16. Primary support of uterus by: strongest by pelvic diaphragm: supply by perineal N S 2-3-4

a) C- A- Lateral transverse cervical ligament cardinal ligament

b) D- Round ligament

c) c. Pubocervial ligament RF for prolapse menopause and grandmultipra

d) broad ligament

17. ischiorectal fossa communicates:

A- A. anteriorly

B- B-posteriorly spread infection to anal canal

C- Inferiorly

D- Anterolaterally

12- posterior fornix is a gate way to:

Cordocentesis, also sometimes called Percutaneous Umbilical Cord Blood Sampling (PUBS), is a diagnostic test which examines blood from the fetus to detect fetal abnormalities.

A- A.Retroperitoneal space

B- B.Vesicouertine pouch

C- C -Retcouterine pouch dougaous pouch

13- 20 years old female come with non-offensive vaginal discharge , cruddy milk, most likely diagnose is:

A. gardenella vaginalis — colorless fishy odor

B. trichmonas vaginalis. — greenish and frothy

C. Candida albicans. — psudohyphea under microscope - yeast — Tx by azoles; MoA: stop ergosterol synthesis

D. Bacterial vaginosis — colorless fishy odor

14- VDRL AND PRP for syphilis testing are;

a. specific — FTA and TP-PA

A. b.non specific — false + with pregnancy and hepatitis; appear only after 3 weeks

B. c. confirmatory

C. d. diagnostic

15- periventricular calcification in TORCH infection commonly associated with: -

b. A-Toxoplasmosis

c. B-Cytomegalovirus

d. C-rubella

e. D-syphilis

22-3year-old child presents at the physician's office with symptoms of coryza, conjunctivitis, low-grade fever, and Koplik's spots. The causative agent of this disease belongs to which group of viruses?

A. Adenovirus

B. Picornavirus — entero viruses. polio, HAV

toga is rubella

C. Orthomyxovirus — influenza

lenti is retro is HIV

D. Paramyxovirus — measles + para influenza + mumps + RSV

23- The following are true about HIV/AIDS except:

A. type c is most common

B. B.HIV 1 not HIV 2 is responsible for current epidemic

C. C. can be transmitted by sexual contact contaminated blood

D. D. can be caused by HBV

24- in late stage of HIV;

A. A. viral load is decreased high

B. B.CD4 cells are depleted

C. C. acquired immunity unaffected immunodeficiency

25- cause of painful genital ulcer;

A. A. herpes simplex virus can cause both painful and painless

B. B. syphilis is painless + lymphogranuloma venereum is painless by chlamydia

C. C.Haemophilusducreyi chancroid associated with pus and inguinal LN adenopathy

26- Infection with hepatitis D occur with HBV a coinfection or in a carrier What test can be used to determine patient with HDV IS HBV carrier?

A. HBsAg active disease s AB is vaccination

B. HBc IgM appear only in acute infection c IgG is late acute and chronic

A. c. HBeAg infectivity and infectious

B. d. HBs IgM

c Ag in liver only not detected
determine latent infection
reinfection when low immunity

27- Infectious mononucleosis, a viral disorder that can be debilitating, is characterized by which of the following statements?

A. It is most prevalent in children less than 14 years old

B. It is caused by a rhabdovirus

IM: present with Resp tract infection with lymphadenopathy

D- The causative pathogen is an Epstein-Barr virus 90%
10% is by CMV

A. d. Ribavirin is the treatment of choice

ass with nasopharyngeal cancer + non-Hodgkin lymphoma + Burkitt lymphoma
invx: viral core antigen PCR

28- most common cause of UTI:

A. E.coli by Pilli

E- b. proteus 2nd MCC

F- c. pseudomonas aerigenosa MC nosocomial infection

29- Purulent urethritis, most commonly caused by:

B. A.N. gonorrhea

C. Chlamydia trachomatis MCC of STD
MCC of PID
Watery whitish discharge

D. Treponema palledium

acute febrile rash and produces disease in immunocompetent children, associated with transient aplastic crisis in SCA persons:

A. A.reobulla
B. B.varecella zoster
C. C.parvovirus only ssDNA ... causes slapped check disease in children Arthritis in adults
D. d.rubella

30- **urine culture is best specimen is from:**

A. early morning urine early morning for sputum sample
B. midstream urine 1st is normal flora

31- **pt c/o of acute hemorrhagic conjunctivitis and blurring of vision what is the cause:**

A. A.cryptovirus
B. B.enterovirus also cause viral meningitis most likely infect the GIT
C. c-barvovirus

32- **The confirmative test for HIV**

A. ELISA
B. b. western blots detect protein
C. c. total blood count
D. d. urine test

33- **What is infection of women reproductive organs that often cause spread from vagina to uterus to fallopian tube or ovary :**

A. A.PID Tx by doxy + metronidazole + ceftriaxone complication: block tubes lead to infertility or partially lead to ectopic pregnancy
B. b.trichomonus vaginalis
C. c-STDs
D. d.UTI

34- **regarding HSV described by which of the following statements?** HSV type 1 angular stomatitis and encephalitis

A. The CNS and visceral organs are usually involved
B. It rarely recurs in a host who has a high antibody titer
C. It can be reactivated by emotional disturbances or prolonged exposure to sun light also by fever
D. Initial infection usually occurs by intestinal absorption of the virus

HSV 1 and 2 latent infection in skin only... (in nerve is VZS). (HSV type 2 also in sacral ganglia)

f. Infection with type 1 virus is most common

35- All of the following is cause of sexual transmitted disease except:

g. A.neissria

h. B.chlamydia

i. C.gardenella

j. D.pseudomonas aeruginosa Nosocomial, UTI, Burn patient
Tx: by potent Ab e.g. Piperacillin

36- which one of the following statements best describes interferon suspected mode of action in producing resistance to viral infection:

a. A.it stimulates cell mediated immunity 1) phagocytosis by cytotoxic T cells and MQs

b. B.it stimulates humeral immunity

c. c. its direct antiviral action related to the suppression of m RNA formation

d. D. its action is related to the synthesis of a protein that inhibits translation or transcription 2) protein inhibit viral replication

37- Mutation if which the following corresponding gene can result in resistant to HIV infection:

A. T cell receptor

B. chemokine receptor CCR5 receptors needed by HIV to make infection

a. c. Fc receptor

e. CD28 receptor

38- nucleosome:

A. contain negatively charged protein

B. Is coiling of each single strand of DNA and histone

C. Is association of each DNA and arginine rich protein (histones)

D. Is combination of DNA and chromatin

39- P53 IS?

A. Oncogen

B. B-cell replication

C. muted in most human cancer

D. D. responsible of DNA repaire = tumor suppressor gene

40- enzyme is specific for liver damage cells is only,

A. A.GPT Glutamic Pyruvic Transaminase aka ALT

B. B.GOT

C. ALP Damage in bile duct

GGT: liver damage by alcohol
A gamma-glutamyl transferase (GGT)

A palm of full-term baby showed single transverse palmer creases the chromosomal abnormality most likely to be?

D. A.45 xy Turner syndrome

E. B.47 xy +18 Edward syndrome .... 13 is Patau syndrome

F. 47 xy +21 Down syndrome

Kleinfelter syndrome is 47 XXY

G. D.69 xxy Partial mole

41- Lactic acidosis caused by deficiency of?

B3 Niacine def: Pellagra
Dermatitis, Dementia and diarrhea

vit A: night blindness and recurrent URT infection and diarrhea

vit K bleeding

A. thymine T in DNA with A

B. B.B1 Thiamine def cause beri beri or Wernicks enchephalopathy

B5 def: burning foot syndrome of prisoners

C. folic acid macrocytic anemia

B12 def cause: Megaloblastic anemia
Homocysteinuria
Decrease methionine (peripheral neuropathy)

D. Biotin

vit C def cause scurvy

42- Which clinical laboratory observation below is suggestive of Hartnup disease (neutral amino acid transport deficiency)? AR inheritance
Tx by high protein diet

A. Burnt-sugar smell in urine

B. High plasma phenylalanine levels

b. Elevated plasma tyrosine and methionine levels

C. High indole derivatives is tryptophan

43- Beriberi? B 1 def

A. not common in alcoholics

D. b.the deficiency of riboflavin

B. affect the nerves and the heart

c. is the combination of diarrhea, dementia and Dermatitis

44- Lipoprotein lipase: for storage enhanced by insulin. move TAG from blood to adipose tissues

A- Hydrolyses cholesteryl ester

B- Is present in the cytosol of adipocytes

C- Is active during starvation

Hormone-sensitive lipase by Glucagon
move TAG from adipose tissue to blood to be used

D- Hydrolyses TAG in chylomicrons or VLDL

from blood to adipose tissue cytosol

45- Pyruvate carboxylase?

A- A-converts acetyl COA to pyruvate

B- require carnitine

C- C-converts pyruvate to oxaloacetate

D- D-is activated by high fructose 1,6 biphosphate

46- Synthesis of glucose from pyruvate by gluconeogenesis in the liver:

A. Occurs exclusively in the liver also in kidney

B. Is inhibited during prolonged fasting stop in starvation

C. Requires participation of Biotin by pyruvate carboxylase

d. Requires lactate as intermediate

47- both glucogenic and ketogenic amino acids except:

e. A.phenylalnine

f. B.tyrosine

g. C.valine only Glucogenic

h. d.thrionine

48- in DNA Watson and crick model:

a. A. thymine base pairs with adenine

b. b. thymine base pairs with cytosine

c. DNA is a single strand

Watson and Crick Theories
1) DNA is polymer of deoxynucleotide = base + sugar (deoxy) + phosphate (phosphodiester bond)
2) C with G and A with T
3) Hydrogen bond by Van Der Waals forces 2 in AT 3 in GC
4) Denaturation occurs by heating
5) Nucleotides are attached by phosphodiester bonds cleaved only by endonuclease
6) DNA is helical double strand with grooves for enzymes

49- the bonds that stabilize the structure of DNA are:

a. A.hydrogen bonds and peptide ponds

b. B.glycosidic bonds and thioester bonds

c. C.phosphodiester bonds and hydrogen bonds

d. d.disulphide bonds and hydrogen bonds

50- why glutamate is important in transfer of ketoacid in transdeamination reaction;

a. A-algha ketoglutrate is abundant in body

C. oxidative deamination in the liver

51- Cholesterol?

A. Normal cell membrane constituent

B. Precursor of sphingolipids

Cholesterol is regulated by LDL to body
HDL from body to liver
HDL: LDL ratio higher is good pt must be fasting
Cholesterol benefits:
1) Bile acid synthesis
2) Steroid hormone synthesis

b. Main source of energy in the body

c. Produced by HMG lyase reductase

52- synthetic nucleotide analogs: with DNA polymerase in PCR

A. Can inhibit DNA synthesis

B. Are always modified purine nucleotides

d. Lack phosphate group

e. Allow replication to proceed at faster rate

53- All of the following are consumed in urea cycle except :

A. aspartate

+ Ammonia + ATP

B. Co2

C. Ornithine

54- Which of the following statement about galactosemia is correct?

A. A-Treatment involves restriction of glucose in the diet

B- A deficiency of glucokinase is the major cause

C- Is treated by elimination of galactose from the diet milk (Glc + Galactose)

B. d- Can be treated by restricting sucrose in the diet maltose is Glc + Glc

Glc + Fructose

55- Arachidonic acid:

A. Is branched fatty acid Unbranched

B. Contains 3 double bond 4 double bonds

C. Is a precursor for eicosanoids Gives PGs and LTXs

e. Contain 5 double bonds

56- the following are purine nucleotides:

f. A.CTP– GDP pyrimidines are C + T + U

g. B.NAD– TDP

D. GMP – ADP A + G

h. D.UDP– GTP

57- During prolonged fasting?

E. A-B-Oxidation of fatty acid is initiated

F. B-glycogenesis is very active

G. C-blood glucose level is maximum

H. D-ketogenesis is maximum main in starvation

58- All the following use ketone bodies oxidation as significant source of energy except:

A. liver

B. kidney

C. RBC no mitochondria

D. heart

59- Which one of the following is not a characteristic of cancer cell?

A. A.Evansion of apoptosis

B. B.hypersensitivity to growth inhibitors signals not sensitive at all

C. c.ability to invasion and metastasis most important in cancer

D. development of sustain angiogenesis

60- excision of small subcutaneous nodule after appendectomy by 28 days microscopic biopsy shows macrophages, collagen, lymphocytes . Which refractile material are seen in nodule and giant cells and surgical complication is? foreign body

A. A.chronic inflammation

B. B-suture granuloma

C. C.abcsess formation

61- raised nodular scar after 2 month of removing wound suture this is?

A. A.keloid

B. B.2ndary union

C. C-orgnization

Stages of wound healing:
hemostasis, inflammation, proliferation (regeneration), granulation tissue and remodeling

62- heart alteration following long untreated HTN?

A. atrophy

A. B.Hyperplasia

B. c.Glycogen storage

C. D.Hypertrophy

- Hypertrophy only: in athletes and heart muscles
- Hyperplasia only: bone marrow in hypoxia
- Both: in gravid uterus
- Endometrium + Breast change in menstrual cycle is hypertrophy only (progesterone)
increase in number is due to (estrogen) in pregnancy

63- predominant cell type in acute inflammation is:

A. A.Neutrophil in acute inflammation and (bacterial infection + antibody (plasma cells))

B. B.monocyte

C. macrophage + lymphocytes in chronic inflammation giant cells in foreign body lymphocytes in viral

D. eosinophil worm and parasite

64- calf muscle affected after long period of immbolization by;

A. A.hypertrophy

B. B.atrophy decrease in cell size

C. c.dystrophy

causes of atrophy
1- long period of immobilization
2- decrease blood supply (ischemia)
3- decrease hormone supply (menopause)
how does atrophy occur? decrease protein content

65- esinophilic staining with loss of nuclei and striation in myocardial tissue?

A. A-coronary artery thrombosis

B. B-viral infection

C. C-chest trauma

coagulative necrosis of heart muscle:
solid organ, eosinophilia, striated muscle

types of necrosis are 6 important with examples

66- tumor markers useful in:

a- diagnosis of Ca

A. b.determining staging TNM

B. assessment of response to treatment

C. d-prediction prognosis + follow up

Tumor markers:
PSA: Prostatic ca
CA 125: Ovarian ca
CA 19-9: Pancreatic ca
CA 15-3: Breast ca
Calcitonin: Medullary carcinoma of thyroid
Catecholamines: Phaeochromocytoma
AFP: Hepatocellular carcinoma
Carcinoembryonic antigen (CEA): Colorectal ca

67- Arterial infarction is least likely to be occur in:

b- intestine 2nd due to anastomosis

c- spleen

d- liver 1st due to dual blood supply

e- kidney

Arterial infarction most likely occurs in the brain
Brain cells cannot handle ischemia for only 30 seconds
Heart cells handle 30 mins
White infarction (ischemic): arterial occlusion or solid organ
Red infraction (hemorrhagic): venous occlusion in non-solid organ

68- antitumor mechanism:

A. Direct killing by NK Cell

B. phagcytosis by lymphocyte

d. Neutralised by AB

e. Phagocytosis by neutrophils

69- metastatic calcification:

a. A.In TB

b. B.Vit D intoxication

c. c.amylodosis

Causes of metastatic calcification
1- MCC is the primary hyperparathyroidism
2- 2nd MCC is cancer and bone metastasis
3- 3rd MCC is sarcoidosis by vit D (all the body)
4- Vit D intoxication (mainly around the kidney)
5- Multiple myeloma

d. d-acute haemorrhgic pancretitis

70- **increased rate of deposition and decrease bone adsorption:** decrease lysis

a. A.increase estrogen
b. B.decrease parathyroid hormone
c. C.increase calcitonin

71- **Which of the following premaligant lesion:**

a. A.hetropia
b. B.hamartoma
c. C.metaplesia
d. d.hypertrophy

the most important premalignant lesion is
Dysplasia (reversible) unlike anaplasia (irreversible)

example in cervix > metaplasia > dysplasia > CIN > Cancer
the most predictable cancer is cervix (stages)

72- **in pheochromocytoma which is medullary carcinoma with increased secretion of epinephrine, this lead to :** Dx by clinical presentation of untreated malignant hypertension

a. A.decreased VMA increase due to epinephrine breakdown
b. B.decreased heart rate increase
c. C.decreased blood pressure increase
d. D.sweat calmy skin

75-8 years old female is able to bend her thumb to touch her forearm, skin pull out about 8 cm, she has difficulty in healing of skin cuts. The underlying disease is caused by defect in:

a. A.collagen
b. B.factor VIII
c. c.antitrypsin
d. d.retinoblastoma protein

Dx is Ehlers-Danlos syndrome
Beighton score to determine hyper ligamentous
Gene Pro-collagen problem

76- **pathology of necrosis in Brain :**

a. A.coagulative necrosis
b. B.liquifactive necrosis
c. C.karyorexis

77- **creatinine kinase is a value test in:**

a. A.muscle disease
b. B.congestive heart failure
c. C.liver cirrhosis

CK MB in heart
CK MM in skeletal muscles

d. D.heamolytic anemia

78- **after femur fracture, the patient complains of SOB and took his blood for investigation , they found that is not clot what is the most likely diagnosis:**

a. A.air embolism

b. B.fat embolism occurs immediately or within 6 hours also in amniotic fluid embolism

c. C.pulmonary thromboembolism due to immobilization, needs time to develop, >72 h

79- **generalized edema in heart failure is due to:**

= anasarca causes: (transudate)
1- increase HP
2- decrease OP

- localized edema causes: (exudate)
1- lymphatic obstruction
2- increase permeability in inflammation

a. A.Lymphatic Obstruction

b. B.Decreased oncotic pressure

c. C.Increased hydrostatic pressure

d. d-acute inflammation increase permeability leading to increase exudate

80- **in septic shock lead to:**

Septic shock is caused by gram-negative endotoxin (lipopolysaccharides) leading to generalized vasodilatation: fever, low BP, and high WBC. (increase COP)

Shock is decreased tissue perfusion
VS septicemia: normal BP
VS Hypovolemic shock (decrease COP)

a. A.generalised vasodilatation

b. B.loss of consciousness

c. c-hypotension

81- **The substance synthesized by fibroblast and connect the cellular integrin to extracellular collagen and to basement membrane**

A- fiberonectin used for connection

B- procollagen

a. c- dermatan sulphate

b. d- hyalouronic acid

82- **Glomerular filteration rate would be increased by:**

A. increase in afferent arteriolar pressure = dilation = increase flow = decrease resistance

B. decrease in efferent arteriolar resistance

GFR= Ux X Vx / Px
normal is 125 ml/min

A- C.increase in the concentration of plasma proteins

B- d.compression of renal capsule

Factors that affect GFR:
1- HP of glomerular cap.: direct relation.
Increase by afferent dilation and efferent constriction
2- OP of glomerular cap.: inverse relation.
Hypoalbunemia increase GFR
3- HP of Bowman's space: inverse relation
Renal obstruction (stone) decreases GFR
4- Permeability of the membrane: Direct
GFR increase by glomerulonephritis
5- Surface area: direct relation. by relaxation or contraction of mesangial cells
6- Changes in renal blood flow: direct relation
7- Sympathetic activation: inverse relation.

83- **In the ECG the p wave denotes:**

A. atrial contraction

B. atrial depolarization

c. ventricular repolarize T wave

ST segment for isoelectric concentration

d. SA node depolarization

84- ST segment elevate in II, III, IVF is represents:

a. A.posterior MI

b. B.anterolateral MI

c. C.inferior MI

85- which substance increased effective renal surface area ;

a. A.ADH

b. B.histamine

c. C.Endothelin

d. D.dopamine by effect on mesangial cells

86- Podocytes associated with which part of urinary tract systems:

a. A.DCT

C. Bowman capsule

b. c.Minorcalycas

c. Major calycas

- Podocytes have negative charge that's why cannot pass protein
- PCT is mainly for reabsorption
- Loop of Henle: countercurrent for osmolarity developing the osmotic gradients needed to concentrate urine.
- CD: hormonal action and acid-base balance

87- vomiting and diarrhea results in:

A. decrease ECF leading to RAAS + ADH activation

= increase ECF osmolarity
= decrease ICF volume due to diffusion

a. B.occurs after delivery

b. c.plasma loss

88- sickling of RBS triggered by

SCA point mutation: Glutamate (polar water soluble) is replaced by Valine (not water soluble) leading to sickling and blood vessel occlusion leading to vaso-occlusive crisis (Hb S)

B. A.increased dehydration, increased p H and decrease oxygenation

C. B.decreased p H and decrease oxygenation

D. decreased dehydration, decreased p H and decrease oxygenation

E. increased dehydration, decreased p H and decrease oxygenation

89- Which of the following hormones act on the anterior lobe of the putaitry to inhibit secretion of prolactin:

A. A.dopamine

B. B.growth hormone

C. c-somatoststin

dopamine analogues examples:
Cabergoline
Bromocriptine

90- after a long period of corticosteroid therapy, the wound healing is delayed due to:

A. A.collagen damage

B. b.fibroblast elaboration

C. c-infection

91- Renal response due to hyperaldosteronism cause :

A. A.increase excretion of bicarbonate

B. increase Hydrogen secretion

due to hypokalemia leading to metabolic alkalosis
(+ HCO3- reabsorption)
unlike hyperkalemia and metabolic acidosis

C. increase K reabsorptiin

c. increase Hydrogen reabsorption

92- About renal circulation :

A. receive 10% of COP 20-25% is RBF

d. B.in exercise COP increase and renal flow increase

e. c.Angiotensin II maintains GFR when renal blood flow increase

93- total peripheral resistance is controlled by

Highest systolic pressure is in renal artery
Highest flow rate in aorta
Lowest flow rate in capillary

B. A.arteriolar tone

C. B-venule Highest capacity of the blood

D. C.external and internal iliac A

COP= HR x SV
BP= COP x PR
Fick's principle is COP= O2 consumption/ A-V

Minute ventilation = tidal volume x respiratory rate
Total lung capacity = TV + IRV + ERV + RV
or = VC + RV

94- baroreceptors:

A. A.its stimulation leads to sympathetic activity

B. B.located in carotid sinus and aortic arch

stimulates by increased BP to Decrease HR
Immediate control of BP

C. C.stimulated by hypoxia

Central chemoreceptors in medulla (by CO2, low PH)
Peripheral chemoreceptors in ao. bodies (by hypoxia)

95- shock is:

A. A.decrease tissue perfusion

B. B-bacterial infection

C. c-microbial in blood

96- ammonia is a good buffer in urine because

A. A.renal produce large amounts of NH3 in chronic acidosis

B. B.renal wall impermeable to NH3

C. c.renal wall permeable to NH4

D. d.reabsorbed in thick ascending loop

97- in exercise man measuring ACTH and cortisol at evening and they found it in high level its occur in :

A. A.cushing

B. B.normally found this result in healthy exercisable man

d. Addison disease

C. d.conns disease

98- compensated respiratory acidosis marked by:

Anion gap is affected by albumin, phosphate and lactate

e. A.increased acidity of the urine

f. B.increased secretion of chloride

g. C.ammonia production by the kidney By increasing HCO-3 reabsorption and H+ excretion (by ammonia)

99- failure to correct hypokalemia is concomitant caused by:

a. A.decrease sodium count

C. decrease magnesium count

b. C.decrease chloride count

100- hypokalemia diagnose clinically mainly by : Hypokalemia in ECG inverted T wave, U wave

A. cardiac arrhythmia

D. B.antagonised digoxin effect

E. C.dierrhea

F. d.hyperreflexes

101- the following statement concerning the defecation are true except:

A. A.the act is often preceded by the entrance of the feces into rectum.

B. B.the anterior abdominal muscles contract

C. C.the internal sphincter contracts and causes evacuation of feces IS relax Autonomic control

D. D.the external anal sphincter and puborectalis relaxes somatic control

102- Two or more group 0f subjects follow for 5 yrs to compare treatment OF sidroblastic aneamia .the most analysis of different to be use is?

A. A-regression analysis T-test for small numbers Z-test for large numbers

B. square test Chi-square test for qualitative data (smoker vs non-smoker)

C. ANOVA

103- Following a large group of cigarette smoker for 10 years for complication like COPD , ca lung…

A. A-cross sectional study

B. B-case control

C. C -cohort study

104- Which is incorrect in sample distribution of sample mean?

stander error = SD/ square root N

A. A-stander error of sample mean decrease as sample increase

B. B-stander error of sample mean is measure different of sample mean in repeated sample

C. C -sample mean is unbiased for true (unknown population

105- Which of the following is correct?

A. A- a large sample always ensure that sample is represent of population

B. B-in random samples the randomization ensure that we have precise and accurate estimate

C- we do not need to randomize if our sample size is sufficiency enogh

D- if all other things are equal,we need alarger sample size

106- reversible inhibition of cycloxgenase in platelet is caused by?

A- A-alprostadil

B- B-asprin Irreversible inhibition of Cox

C- C-paracetamol

D- D-ibuprofien + Indomethacin + Mefenamic acid

107- Cardiotoxicity limits the clinical usefulness of which one of the following antitumor antibiotics :

Cisplatin SE: deafness
Cyclophosphamide SE: hemorrhagic cystitis

A- A.Doxorubicin

c. Bleomycin SE: Pulmonary fibrosis

d. Dactinomycin

e. Plicamycin

Teratogenic Abx
- Tetracycline: Discoloration of teeth
- Ciprofloxacin: Cartilage dysgenesis
- Chloramphenicol: Gray baby syndrome + aplastic anemia
- Sulfonamides: Kernicterus anemia

108-A nucleophilic attack on deoxyribonucleic acid which cause disruption of base pairing occurs as a result :

A) Cyclophosphamide

c) Thioguanine

Chemotherapeutic agents
1) Alkylating agents e.g. Cyclophosphamide attack in S phase
2) Alkaloids e.g. Vincristine attack in M phase
3) Antifolate e.g. Methotrexate

d) Prednisone

e) Methotrexate Antifolate

109-19 yrs female diagnosed with TB. before prescribing treatment you should be aware of that one antituberculus induce cytochrome P450 enzyme in the liver :

a) A.rifampicin+ anti epileptics + chronic alcohol

b) B.isoniazid

c) c.pyrazinamide

d) d.ethambiotol

Cyt P450 inhibitors:
increase drug toxicity
- Chloramphenicol
- Antiacid
- Acute alcohol

110- **Which of the following antiviral agents is a purine nucleoside analogue that has shown promise with Lassa fever, influenza A and B, and respiratory syncytial virus (RSV)?**

A. Amantadine in influenza only

B. Rimantadine in influenza only

C. Ribavirin

e. Acyclovir

111- **drug use to treat angina consistently cause reduction of HR and prevent vasopasmodic angina ?**

f. A.nitroglycerate

g. B.propranolol

h. C.verapamil calcium channel blocker

i. d.isosorbide dinitrate

112- **Coombs test is positive and associated with hemolytic anemia in ?**

a. A.methyledopa in 2% of patients taking the drug

b. B.captopril

c. C.clonidine

113- **atropine overdose lead to:** antimuscarinic

a. A.bronchoconstrection

b. B.increase HR cause try mouth

c. C.increased gastric secretion

d. D.pupillary constriction

114-45 years old treated for ovarian cancer developed difficulty hearing:

a. A.paclitaxil
b. B.bleomycin
c. C.cisplatin

Hypersensitivity reactions:
- Type I: immediate, mediated by IgE antibodies.
- Type II: cytotoxic reaction mediated by IgG or IgM. (acute graft rejection)
- Type III: reaction mediated by immune complexes. SLE
- Type IV: delayed reaction mediated by cellular response (chronic graft rejection)
- Type V: MG, Gravis disease (antibody cause specific function)

115- asthma induced by:

a. A.type I hypersensitivity
b. B.adrenergic agonists
c. c.beta 1-antagonst

116- benzodiazipine action:

a. A.enhance GABA B neurotransmitters
b. B.enhance GABA A in spinal cord through chloride channels
c. C.enhanced Cl channels that act on GABA neurotransmitters

117- amiloride potent diuretic action by:

A. aldesteron antagonist Spironolactone
a. B.acts on ENaC
b. C.Na -K blocker Amiodarone

118- drug to used in lowering blood pressure, analegesic and spasmolytic And given intrthecally:

B. A.tomolol
C. B.clonidine centrally acting, central alpha 2 agonist
D. C.morphine
E. D.nitropuriside

119- primary mechanism of flurosemide:

Carbonic anhydrase inhibitors e.g. Acetazolamide

A. A.inhibition of sodium potassium ATPase
B. B.inhibtion of Na k cl co- transorter
C. c.inhibtion of Na Cl co-transporter Thiazide diuretics
D. e-inhibtion of Cl transporter

Carbonic anhydrase inhibitors e.g. Acetazolamide act on PCT
Loop > Thiazide + Furosemide
DCT > K+ sparing

120- anaphactic shock treatment is by

A. A.epinephrine = Adrenaline
B. B-chorpheniramine maleat

C. C-atropine

D. D-hydrocortiso